9. Warn your patient. Warn your patient right before inserting the needle. Inserting without warning can shock your patient, and they may move, which could cause you to accidentally injure yourself or them.
10. Insert the needle. Use the thumb on your nondominant hand to anchor the vein 1 to 2 inches below the venipuncture site. Hold the tube holder in your dominant hand, positioned at the angle that best matches the depth of the vein. Insert the needle swiftly, with the bevel facing up, and in one fluid motion into the area where you have identified the location of the vein, maintaining the chosen angle. When you insert the needle into the arm, you may initially feel some resistance and then a slight "give" as it enters the vein. What you are feeling is the puncturing of the skin, then the wall of the vein. The slight give is when you enter the middle of the vein, also called the ***lumen***. Once you enter the lumen, you will stop inserting the needle, because this is where you want the needle settled to begin blood collection.
11. Anchor the tube holder. Continue to hold the tube holder at the angle the needle was inserted. Press your knuckles lightly down onto the arm to help reduce movement during tube change.
12. Insert the first tube. Insert the first tube, confirming that you are adhering to the order of draw. Allow the tube to fill completely.
13. Release the tourniquet and open fist. After establishing blood flow, release the tourniquet and ask the patient to relax and open their fist.
14. Remove the first tube. Gently remove the tube from the hub.
15. Invert the tube. Mix the additive into the blood by inverting the tube as soon as it is filled. Inverting the filled vacuum tubes assists in the mixing of the additives that are in the tube with the patient's blood to help preserve the sample and provide the most accurate test results.
16. Insert the next tube. Insert the second tube.
17. Continue to fill tubes until the order is complete, inverting after each one is filled.

Fill Tubes Completely

Every tube you collect must be filled completely. You will know that it is filled when the tube stops filling. The additive in the tube and the methods of testing in the laboratory require that each tube is filled to the proper amount to help provide the most accurate test results. Some tubes, such as the light blue tube, will be rejected by the laboratory if they are not filled completely.

Color of Blood

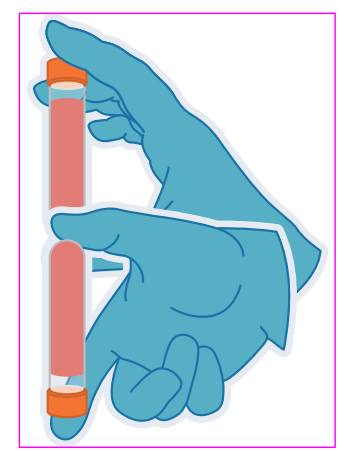
The blood collected during venipuncture is venous blood. It is dark red to reddish-brown in color. Blood from an artery is bright cherry red. Observe the color of blood flowing into the tubes, confirming that it is venous blood. If the blood is ever bright cherry red, stop the collection immediately and apply pressure to the site.

lumen. Middle of the vein; target for venipuncture.

Conversation With Your Patient

As blood fills into the tubes, especially when you are filling a large number of tubes, talking to your patient helps not only to keep up the blood flow but also to determine your patient's condition during the collection. Avoid discussing politics, religion, or any other potentially emotional topics.

Properly Inverting Vacuum Tubes



Carefully rock the tube, making a complete inversion by turning the tube completely bottom up and returning to the stopper-up position. Each one of these inversions is considered one time. Every tube should be mixed, but not every tube is mixed the same number of times.